

Current State of Telehealth & What's Going on After 10/1/2025 (and the Final Rule)

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Today's Faculty: SONAL PATEL



SONAL PATEL

Sonal has over 15 years of experience understanding the art of business medicine. She is a nationally recognized thought-leader, speaker, author, creator, and consultant. She is the founder of SP Collaborative, LLC.

Sonal is an active member with the AAPC®, the American Medical Billing Association (AMBA®), Health Care Compliance Association (HCCA®), NAMAS®, and Practice Management Institute (PMI®).

Sonal is currently on the BC Advantage Editorial Board. Sonal is also a published author in numerous industry publications, including AAPC's *Healthcare Business Monthly* and *Knowledge Center Blog*, *Lexology*, *JD Supra*, as well as two chapters in HCCA's 2021 edition of the *Complete Healthcare Compliance Manual*.

She serves as the creator and host of her weekly Paint The Medical Picture Podcast series which is currently in its fifth year.

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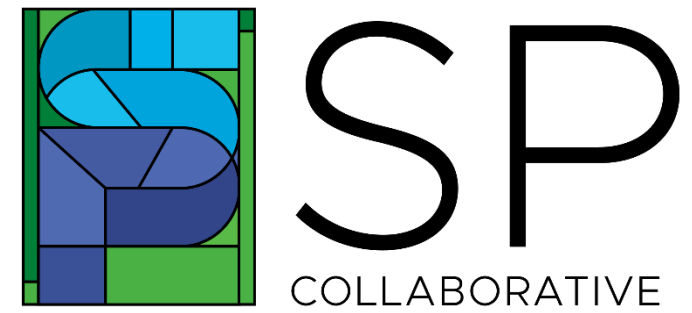
Current State of Telehealth & What's Going on After 10/1/2025 (and the Final Rule)

A Webinar for CME Avenue on November 5, 2025

by Sonal Patel, BA, CPMA, CPC, CMC, ICDCM



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- SP Collaborative recognizes the beauty beneath the art and science of medicine.
 - We provide answers to your coding questions in “Sonal Says”
 - We perform Quality Assurance Audits
 - We provide Content Creation
 - We offer Collaborative Solutions
 - We deliver educational webinars and onsite presentations



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Learning Objectives



Learning Objectives

Government Shutdown

Current State of Telehealth – as of November 4, 2025

Medicare and Telehealth – as of November 4, 2025

What is Permanent?

States and Telehealth – as of November 4, 2025

Billing Pitfalls

Telehealth Under Scrutiny

Other Payors

Final Thoughts



Government Shutdown



Government Shutdown





The Government Shutdown

October 1, 2025

- On October 1, 2025, the federal government of the United States began a shutdown at 12:01 a.m. as a result of congressional failure to pass appropriations legislation for the 2026 fiscal year, which began that day. The shutdown resulted from partisan disagreements over federal spending levels, foreign aid rescissions, and health insurance subsidies.

October 25, 2025

- The shutdown resulted in the furlough of roughly 900,000 federal employees and left another 2 million working without pay. While essential services such as Medicare, Medicaid, and the Transportation Security Administration continued, many agencies, including the National Institutes of Health, the Centers for Disease Control and Prevention, and the WIC program, faced partial or full suspensions of operations.



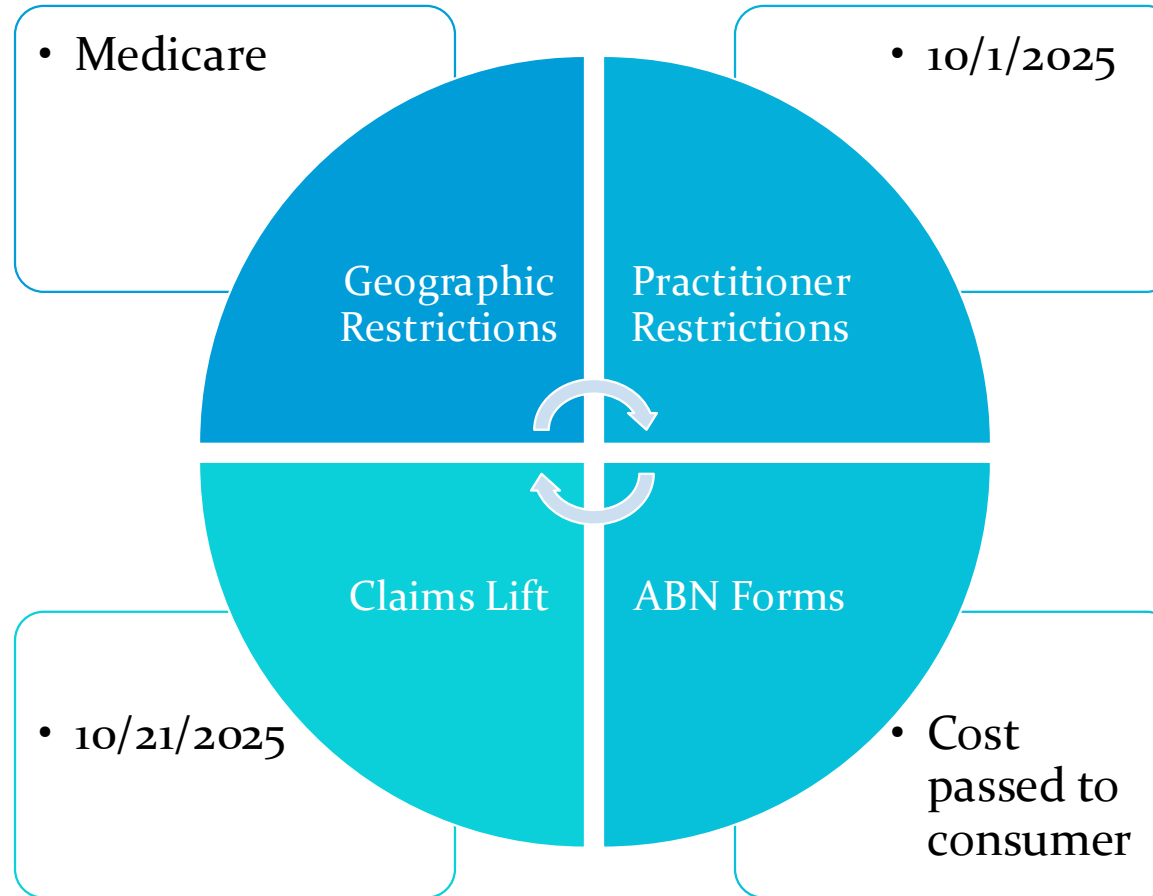
Current State of Telehealth



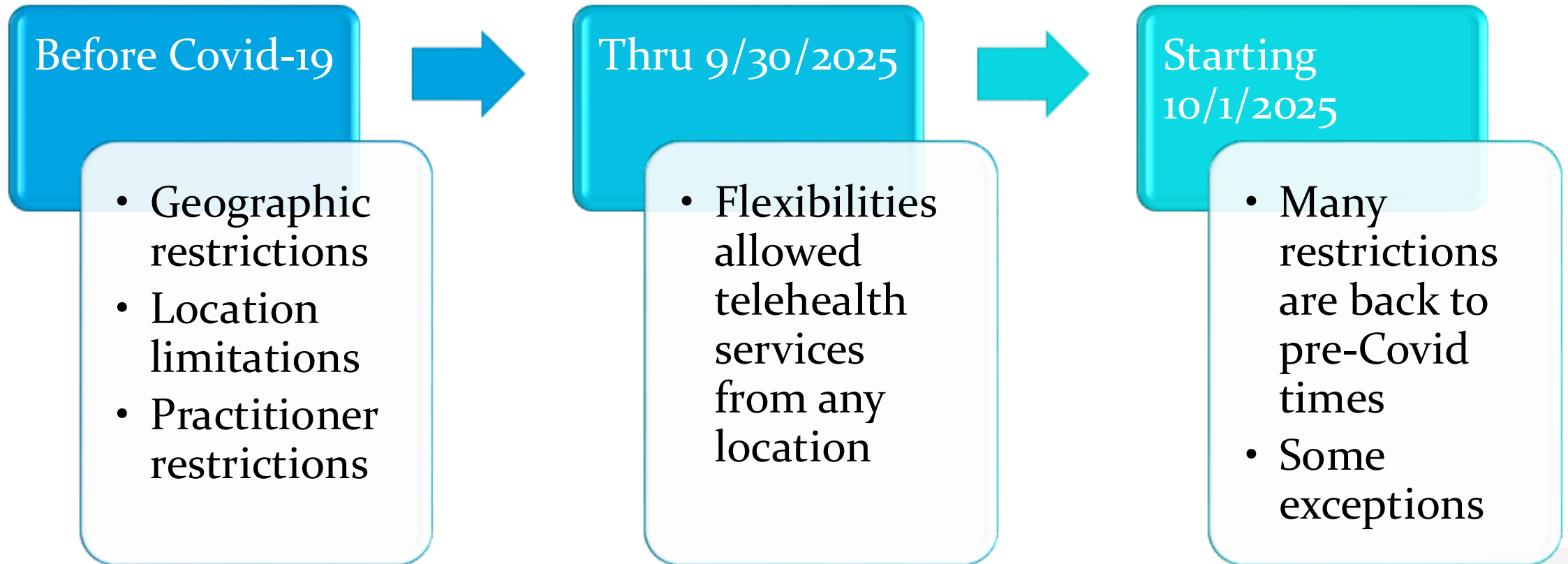
Help! Now What?



Current State of Telehealth - Medicare



Telehealth Timeline





Medicare and Telehealth



What does Medicare Say – November 4, 2025?

10/1 Update

Telehealth FAQ Calendar Year 2025

Q1: Do Medicare beneficiaries need to be located in a rural area and in a medical facility in order to receive Medicare telehealth services?

A1: After September 30, 2025, except for behavioral health services, beneficiaries will generally need to be in a medical facility and in a rural area to receive Medicare telehealth services.

Q2: Are there any restrictions on the types of practitioners who can furnish Medicare telehealth services?

A2: After September 30, 2025, physical therapists, occupational therapists, speech-language pathologists, and audiologists can no longer furnish Medicare Telehealth services.

Q3: Can outpatient therapy, diabetes self-management training and medical nutrition therapy services be furnished remotely by hospital staff to beneficiaries in their homes?

A3: CMS is continuing to align payment policies for outpatient therapy services, diabetes self-management training and medical nutrition therapy services furnished remotely by hospital staff to beneficiaries with policies for Medicare telehealth services. Therefore, after September 30, 2025, hospitals may no longer bill for these services when furnished remotely by hospital staff to beneficiaries in their homes.

- **Rules for Reimbursement:**
- Telehealth has its own Benefit Category – Chapter 15 of the Medicare Benefits Policy Manual
- Telehealth is recognized in Chapter 12 of the Medicare Claims Processing Manual

What does Medicare Say – November 4, 2025?

Geographic Restrictions

- Patient *Must Generally Be in Rural Area Facility, FQHC or RHC

Practitioner Restrictions

- PT, OT, SLP, Audiologist Cannot Perform

What does Medicare Say – November 4, 2025?

POS 02

- Telehealth Provided Other than in Patient's Home

POS 10

- Telehealth Provided in Patient's Home (which is a location other than a hospital or other facility where the patient receives care in a private residence).

What does Medicare Say – November 4, 2025?

Distant Site

- Distant site practitioners may continue to use their currently enrolled practice location instead of their home address when providing Medicare telehealth services from their home

What does Medicare Say – November 4, 2025?

Audio-Only

- Physicians and practitioners may use audio-only communication technology for any telehealth service furnished to a patient in their home, provided that the furnishing practitioner is technically capable of using audio-video communication technology and that the beneficiary is not capable of or does not consent to using audio-video communication technology. Audio-only can be used for both new and established patients.

What does Medicare Say – November 4, 2025?

POS 02 –
Telehealth other
than home

POS 10 – Patient
at Home

Modifier 95 –
Facility Claims

Q3014 –
Originating Site
Facility Fee

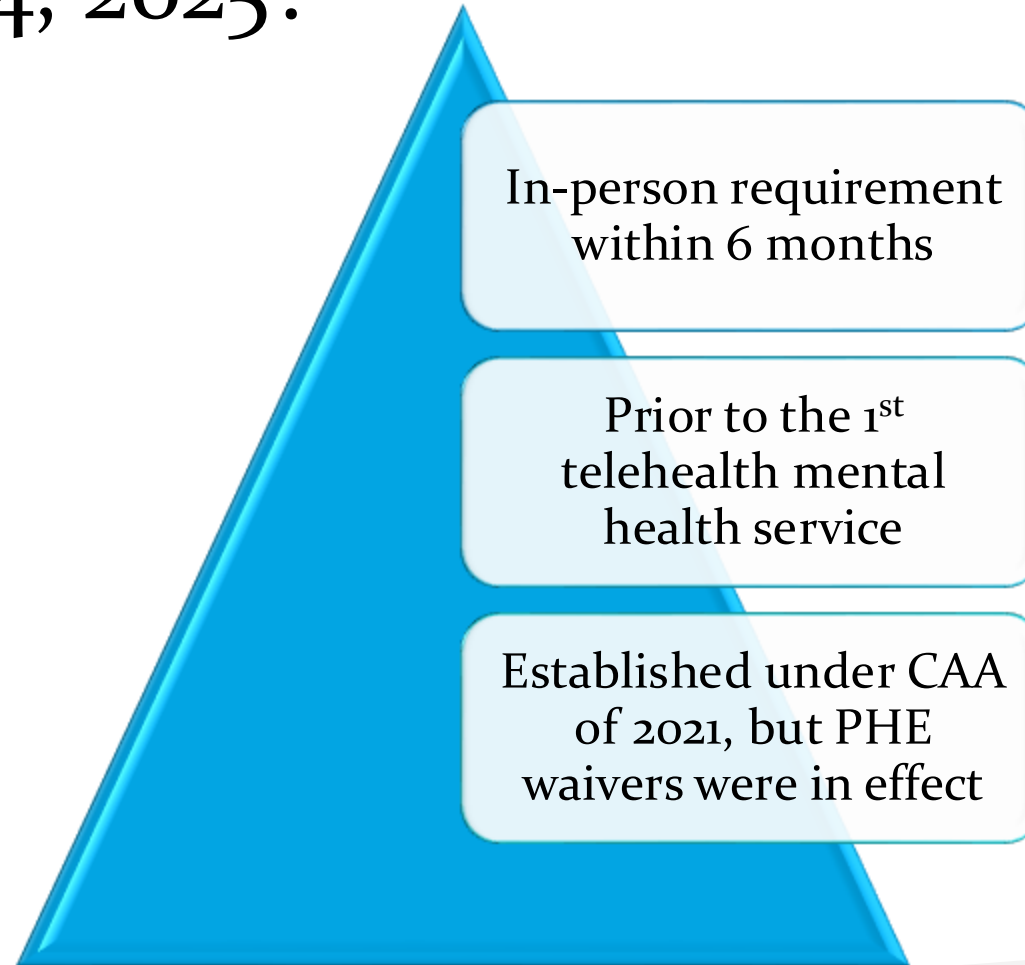
List of Covered
Services



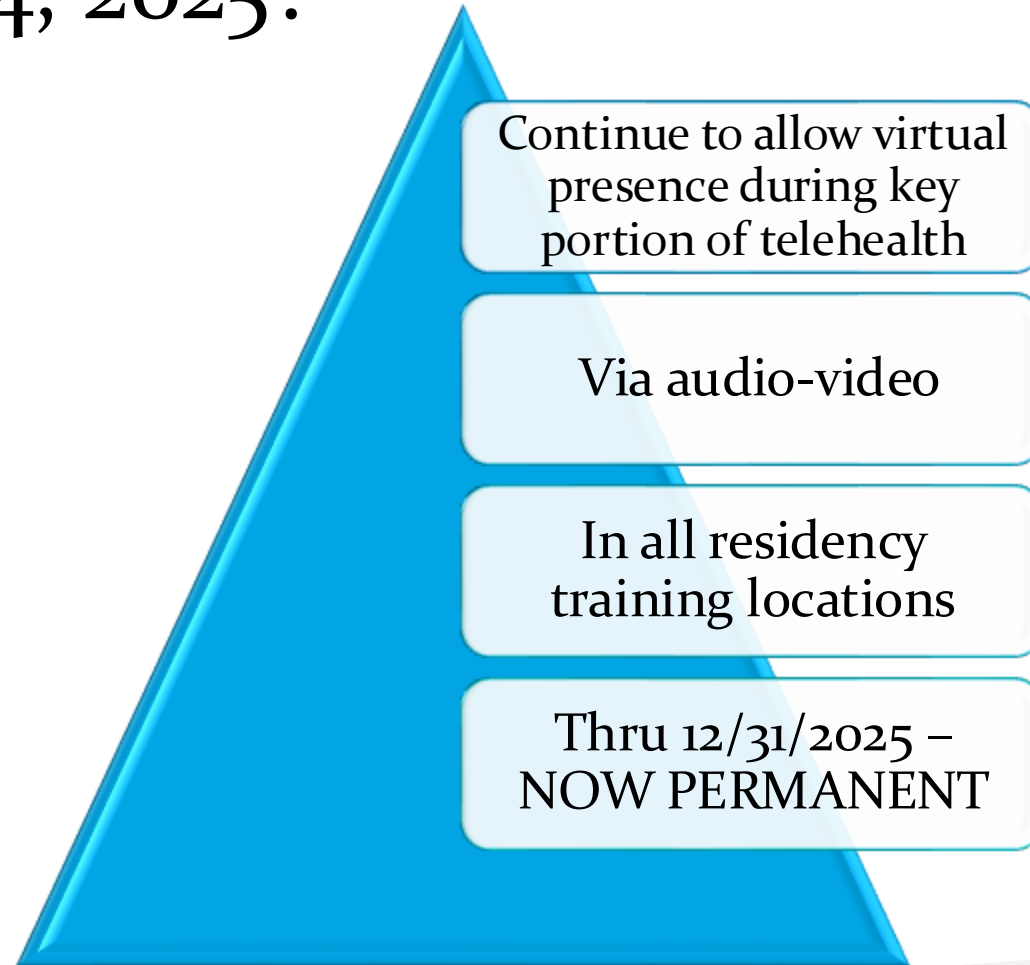
Starting October 1, 2025, the statutory limitations that were in place for Medicare telehealth services before the COVID-19 PHE will retake effect for most telehealth services. These include:

- Geographic restrictions
- Location restrictions on where you can provide services
- Limitations on the scope of practitioners who can provide telehealth services

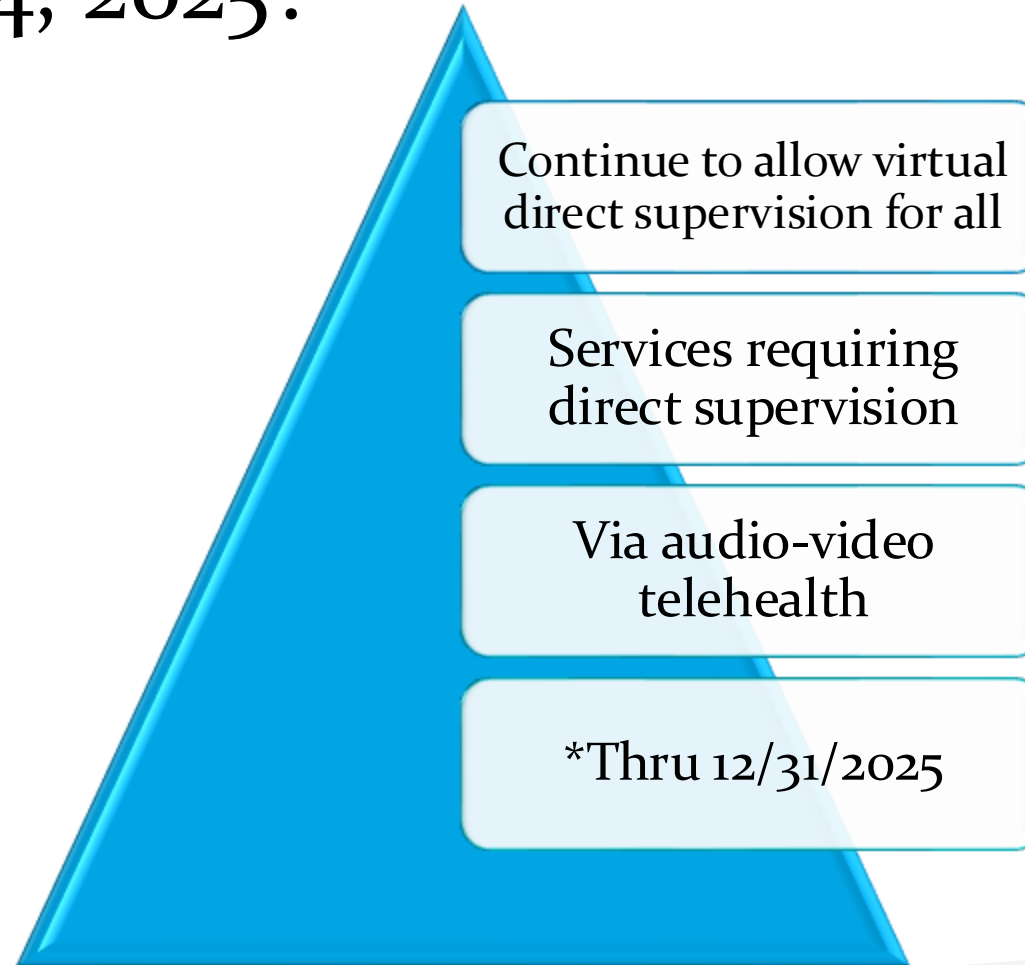
What about Medicare & Behavioral Health – November 4, 2025?



What about Medicare & Teaching Physicians – November 4, 2025?

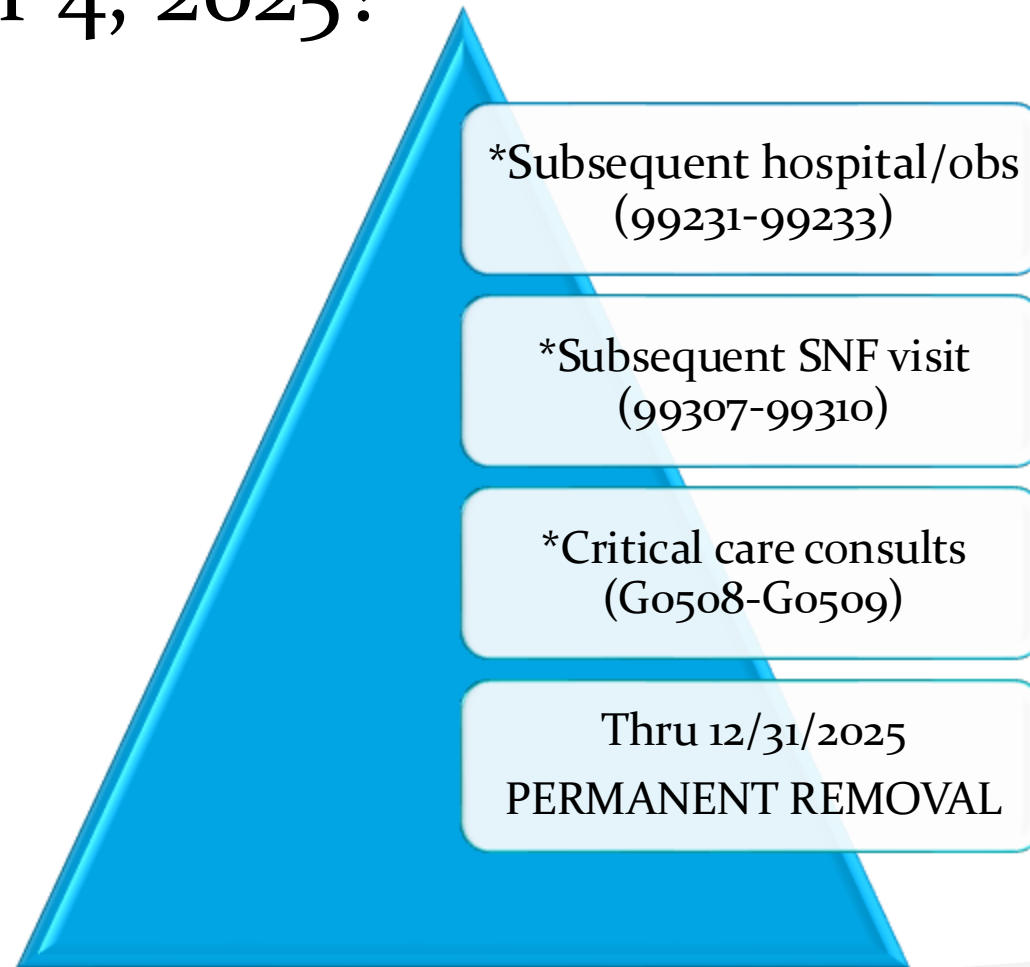


What about Medicare & Supervision – November 4, 2025?

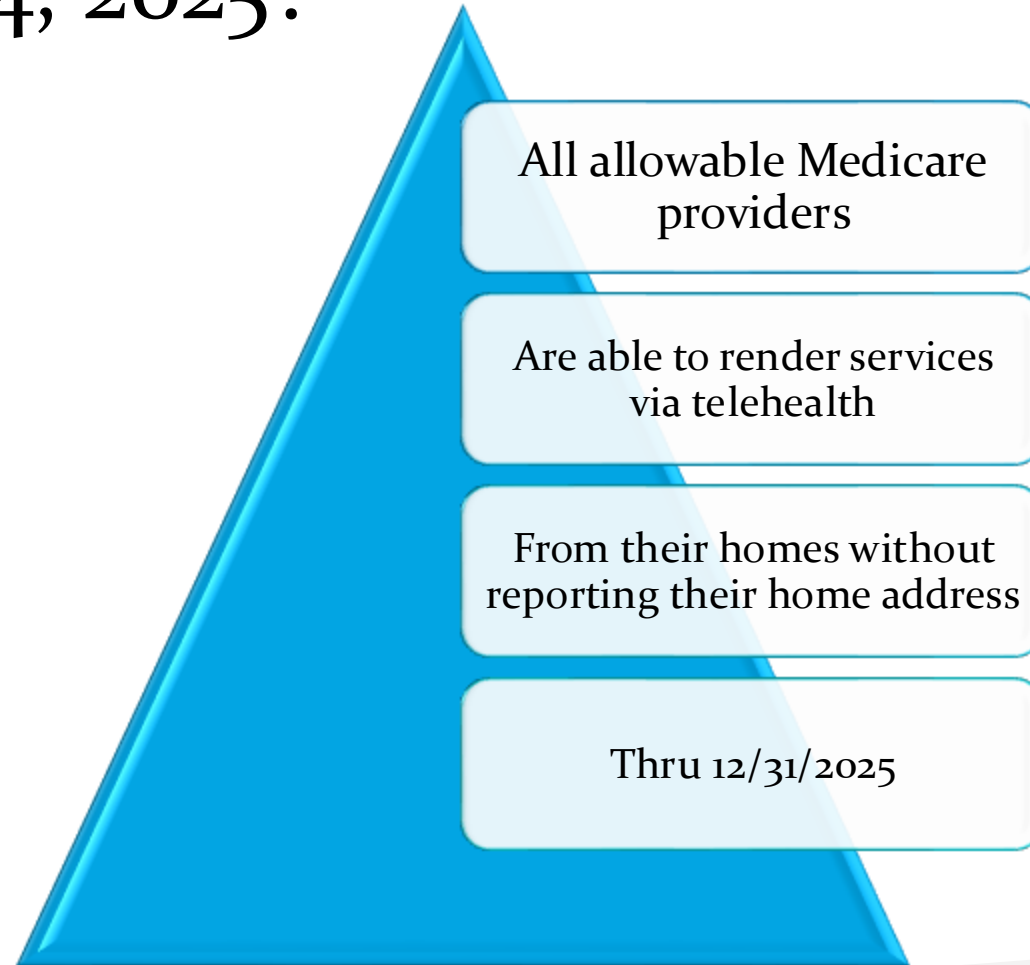


What about Medicare & Frequency Limitations

– November 4, 2025?



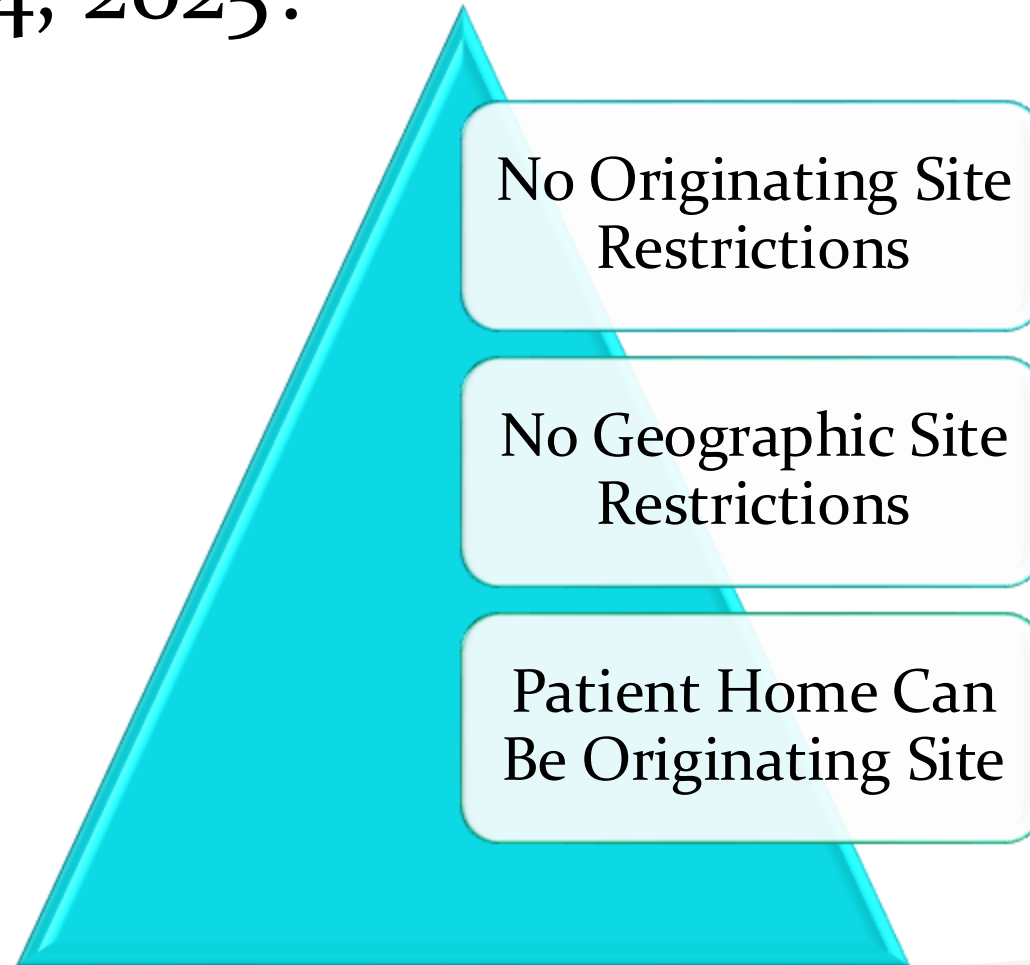
What about Medicare & Provider's Home – November 4, 2025?



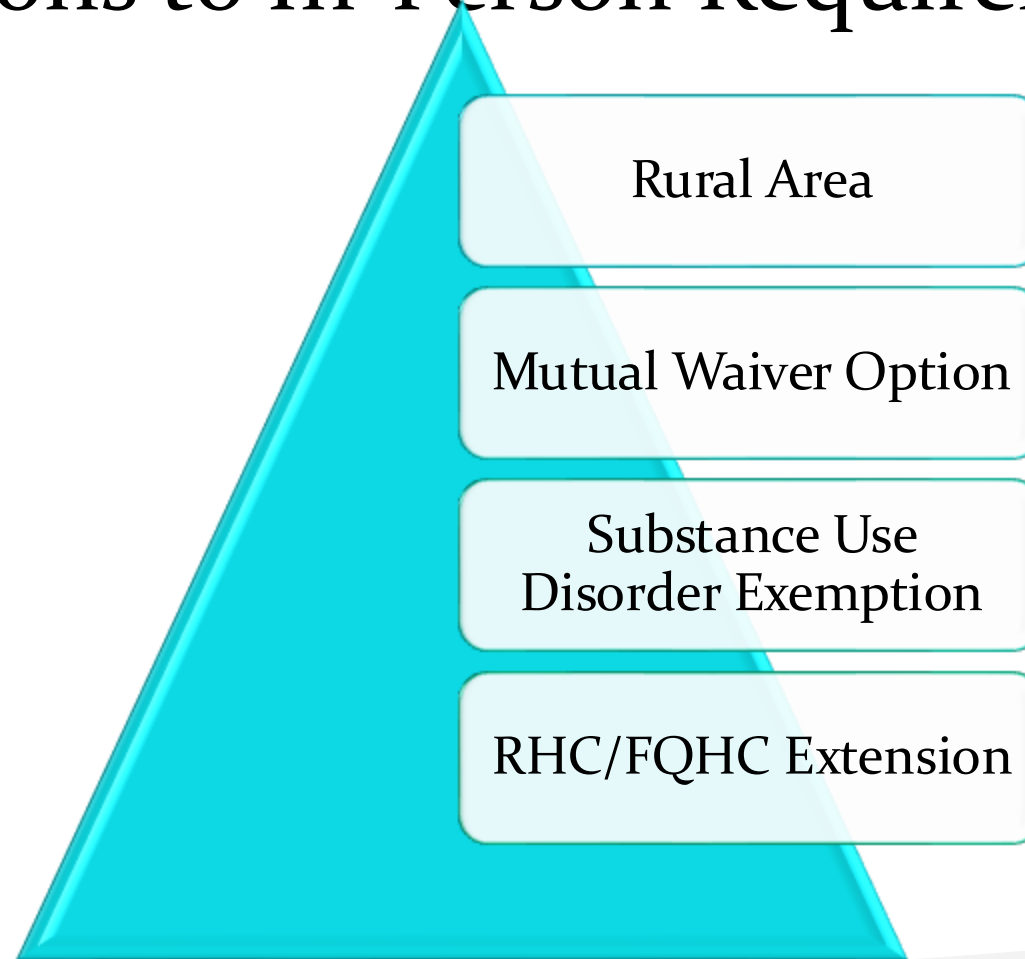


What is Permanent?

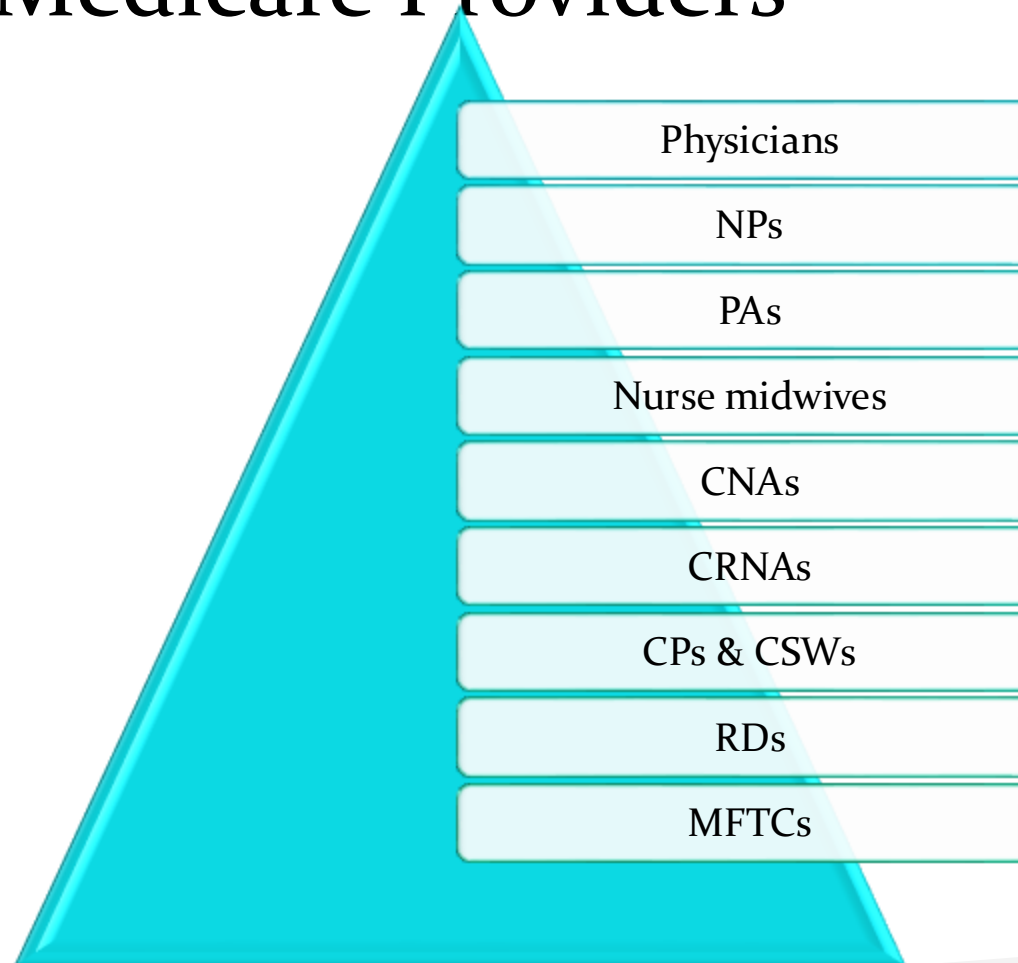
What about Medicare & Behavioral Health – November 4, 2025?



Key Exceptions to In-Person Requirements



Permanent Medicare Providers



Medicare Telehealth Services – 282

	B	C	D	E	F
	FINALIZED LIST OF MEDICARE TELEHEALTH SERVICES				
	HCPCS	Short Desc	Finalized A	Category	
1	0362T	Bhvid supr	Maintain	provisional	
2	0373T	Adapt bhv t	Maintain	provisional	
3	0591T	Hlth&wb c	Maintain	provisional	
4	0592T	Hlth&wb c	Maintain	provisional	
5	0593T	Hlth&wb c	Maintain	provisional	
6	77427	Radiation t	Maintain	provisional	
7	90785	Psytx comp	Maintain	permanent	
8	90791	Psych diag	Maintain	permanent	
9	90792	Psych diag	Maintain	permanent	
10	90832	Psytx w pt	Maintain	permanent	
11	90833	Psytx w pt	Maintain	permanent	
12	90834	Psytx w pt	Maintain	permanent	
13	90836	Psytx w pt	Maintain	permanent	
14	90837	Psytx w pt	Maintain	permanent	
15	90838	Psytx w pt	Maintain	permanent	
16	90839	Psytx crisis	Maintain	permanent	
17	90840	Psytx crisis	Maintain	permanent	
18	90845	Psychoana	Maintain	permanent	
19	90846	Family psy	Maintain	permanent	
20	90847	Family psy	Maintain	permanent	
21	90853	Group psy	Maintain	permanent	
22	90875	Psychophy	Maintain	provisional	
23	90901	Biofeedba	Maintain	provisional	
24	90951	Esrd serv 4	Maintain	permanent	
25	90952	Esrd serv 2	Maintain	permanent	

A	B	C	D	E
256	G0442	Annual alc	Maintain	permanent
257	G0443	Brief alcoh	Maintain	permanent
258	G0444	Depressio	Maintain	permanent
259	G0445	High inten	Maintain	permanent
260	G0446	Intens beh	Maintain	permanent
261	G0447	Behavior c	Maintain	permanent
262	G0459	Telehealth	Maintain	permanent
263	G0506	Comp asse	Maintain	permanent
264	G0508	Crit care te	Maintain	permanent
265	G0509	Crit care te	Maintain	permanent
266	G0513	Prolong pre	Maintain	permanent
267	G0514	Prolong pre	Maintain	permanent
268	G2086	Off base of	Maintain	permanent
269	G2087	Off base of	Maintain	permanent
270	G2088	Off base of	Maintain	permanent
271	G2211	Complex E	Maintain	permanent
272	G2212	Prolong ou	Maintain	permanent
273	G0248	Home INR	No Additio	provisional
274	G3002	Chronic pa	Maintain	permanent
275	G3003	Addition 15	Maintain	permanent
276	G9685	Acute nurs	Maintain	provisional
277	G0539	Initial care	Addition	provisional
278	G0540	train for ca	Addition	provisional
279	G0541	no pt prese	Addition	provisional
280	G0542	Care train	Addition	provisional
281	G0543	Group train	Addition	provisional
282	G0560	Safety plan	Addition	permanent

Claims Hold Lift – as of October 21, 2025



- CMS instructed all Medicare Administrative Contractors (MACs) to lift the claims hold and process claims with dates of service of October 1, 2025, and later for certain services impacted by select expired Medicare legislative payment provisions passed under the Full-Year Continuing Appropriations and Extensions Act, 2025 (Pub. L. 119-4, Mar. 15, 2025). This includes claims paid under the Medicare Physician Fee Schedule, ground ambulance transport claims, and Federally Qualified Health Center (FQHC) claims. This includes telehealth claims that CMS can confirm are definitively for behavioral and mental health services. CMS has directed all MACs to continue to temporarily hold claims for other telehealth services (i.e. those that CMS cannot confirm are definitively for behavioral and mental health services) and for acute Hospital Care at Home claims.

Claims Hold Lift – as of October 21, 2025



- Beginning October 1, 2025, for services that are not behavioral health services, many of the statutory limitations on payment for Medicare telehealth services that were, in response to the COVID-19 Public Health Emergency, lifted, and subsequently extended, through legislation again took effect. These include prohibition of many services provided to beneficiaries in their homes and outside of rural areas, and hospice recertifications that require a face-to-face encounter. In the absence of Congressional action, practitioners who choose to perform telehealth services that are not payable by Medicare on or after October 1, 2025, may want to evaluate providing beneficiaries with an Advance Beneficiary Notice of Noncoverage (ABN). Further information on use of the ABN, including ABN forms and form instructions: <https://www.cms.gov/medicare/forms-notices/beneficiary-notices-initiative/ffs-abn>. Practitioners should monitor Congressional action and may choose to hold claims associated with telehealth services that are currently not payable by Medicare in the absence of Congressional action.



ABN Forms – as of October 21, 2025

- Practitioners who choose to perform telehealth services that are not payable by Medicare on or after October 1, 2025, may want to evaluate providing beneficiaries with an Advance Beneficiary Notice of Noncoverage (ABN).
- Remember, the cost-share would fall directly to the patient



ABN Forms – as of October 21, 2025

- ABN Instructions:
- The ABN is a notice given to beneficiaries in Original Medicare to convey that Medicare is not likely to provide coverage in a specific case. “Notifiers” include:
 - Physicians, providers (including institutional providers like outpatient hospitals), practitioners and suppliers paid under Part B (including independent laboratories);
 - Hospice providers and religious non-medical health care institutions (RNHCIs) paid exclusively under Part A; and
 - Home health agencies (HHAs) providing care under Part A or Part B



ABN Forms – as of October 21, 2025

- These healthcare providers and suppliers must complete the ABN as described below in order to transfer potential financial liability to the beneficiary, and deliver the notice prior to providing the items or services that are the subject of the notice.
- Medicare inpatient hospitals and skilled nursing facilities (SNFs) use other approved notices for Part A items and services when notice is required in order to shift potential financial liability to the beneficiary; however, these facilities must use the ABN for Part B items and services.



ABN Forms – as of October 21, 2025

- The ABN must be reviewed with the beneficiary or his/her representative and any questions raised during that review must be answered before it is signed. The ABN must be delivered far enough in advance that the beneficiary or representative has time to consider the options and make an informed choice. Employees or subcontractors of the notifier may deliver the ABN. ABNs are never required in emergency or urgent care situations. Once all blanks are completed and the form is signed, a copy is given to the beneficiary or representative. In all cases, the notifier must retain a copy of the ABN delivered to the beneficiary on file.
- The ABN may also be used to provide notification of financial liability for items or services that Medicare never covers. When the ABN is used in this way, it is not necessary for the beneficiary to choose an option box or sign the notice.



States and Telehealth

States and Telehealth

The State Focus:

Widely different – found in statutes or state codes

Acceptable modalities
(synchronous, asynchronous, remote patient monitoring)

Practitioner types

Patient to practitioner relationship

Out of state practitioners

The State Payor Focus:

Private/commercial & state policies

Coverage limitations in policies for both

Reimbursement – modifiers, place of service

Eligibility of practitioners



States and Telehealth

The State Focus:

Widely different – found in statutes or state codes

Ohio – OAC 4725-25-01

Illinois – 225 ILCS 150

The State Payor Focus:

Private/commercial & state policies

BUCAs will all be updating their policies after 10/1/2025

But they have each allowed for telehealth services



States and Telehealth

The State Focus:

Ohio – OAC 4725-25-01 : Telehealth communication

(1) "Telehealth services" means health care services provided through the use of information and communication technology by a health care professional licensed in Ohio, within the professional's scope of practice, who is located at a site other than the site where the patient is receiving the services or the site where another health care professional with whom the provider of the services is formally consulting regarding the patient is located.



States and Telehealth

The State Focus:

Illinois – 225 ILCS 150 : Telehealth act

"Telehealth services" means the evaluation, diagnosis, or interpretation of electronically transmitted patient-specific data between a remote location and a licensed health care professional that generates interaction or treatment recommendations. "Telehealth services" includes telemedicine and the delivery of health care services, including mental health treatment and substance use disorder treatment and services to a patient, regardless of patient location, provided by way of an interactive telecommunications system, asynchronous store and forward system, remote patient monitoring technologies, e-visits, or virtual check-ins.

States and Telehealth



Ohio

OVERVIEW

DEFINITIONS

LIVE VIDEO

STORE-AND-FORWARD

REMOTE PATIENT MONITORING

EMAIL, PHONE & FAX

CONSENT REQUIREMENTS

OUT OF STATE PROVIDERS

MISCELLANEOUS

Last updated 06/09/2025

Overview

Ohio Medicaid reimburses for live video telemedicine. Store-and-forward is only reimbursed for certain communication technology-based service codes. Certain remote physiologic monitoring codes are reimbursed along with FQHCs being reimbursed through the FFS under clinic provider type 50 for remote patient monitoring. Audio-only is incorporated into the definition of telehealth in OH Medicaid and is reimbursed for certified community behavioral health centers for CPST and SUD case management. It is also reimbursed through certain telehealth service codes.



States and Telehealth



Illinois

OVERVIEW

DEFINITIONS

LIVE VIDEO

STORE-AND-FORWARD

REMOTE PATIENT MONITORING

EMAIL, PHONE & FAX

CONSENT REQUIREMENTS

OUT OF STATE PROVIDERS

MISCELLANEOUS

Last updated 06/25/2025

Overview

IL Medicaid reimburses for live video telemedicine and telepsychiatry services for specific providers as well as certain Interprofessional Consultation CPT codes that are billable for psychiatric services. IL Medicaid will provide reimbursement for home uterine monitoring, continuous glucose monitoring, and some audio-only services.

The IL Association of Medicaid Health Plans has created a comprehensive [billing manual for IL Medicaid Managed Care](#) that includes some information on telehealth policy as it applies to these health plans.



States and Telehealth

Compare state policies

TOPIC

1 Choose a topic... ▾

⋮

JURISDICTIONS

2 Choose a jurisdiction or click on the map... ▾

⋮

3 Choose a jurisdiction or click on the map... ▾

COMPARE

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Virtual visits

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Billing Pitfalls

Billing Pitfalls

Medicare	Other Payors
Consent to telehealth	Current payor policy
HIPAA-compliant platform	Variance in states
Modifiers	Modifiers
Place of service	Place of service
Documentation	Documentation
Program Integrity Risks	SIU



Modifiers

95	Synchronous telemedicine service rendered via real-time interactive audio and video telecommunications system.
93	Synchronous telemedicine service rendered via telephone or other real-time interactive audio-only telecommunications system. (used with Appendix T codes in CPT)
FQ	A telehealth service was furnished using real-time audio-only communication technology.
FR	A supervising practitioner was present through a real-time two-way, audio/video communication technology.
GO	Used to identify telehealth services furnished for purposes of diagnosis, evaluation, or treatment of symptoms of an acute stroke.
GQ	Asynchronous telehealth service (not used unless you are in Alaska or Hawaii)



Guidelines – Medicare Prefers E/M Codes for Telehealth (not 98000 series)

Synchronous, real-time interactive encounters between physician and other qualified health care professional (QHP)

Level selection based on Medical Decision Making (MDM) or total time

These codes are used to report synchronous, real-time, audio-video interactive telecommunication services for a new patient. A medically appropriate history and physical examination, as determined by the treating provider, should be documented.



Guidelines – Medicare Prefers E/M Codes for Telehealth (not 98000 series)

We do not believe that there is a programmatic need to recognize the audio/video and audio-only telemedicine E/M codes for payment under Medicare. We are proposing to assign CPT codes 9X075-9X090 a Procedure Status indicator of “I”, meaning that there is a more specific code that should be used for purposes of Medicare, which in this case would be the existing office/outpatient E/M codes currently on the Medicare telehealth services list when billed with the appropriate POS code to identify the location of the beneficiary and, when applicable, the appropriate modifier to identify the service as being furnished via audio-only communication technology.

Synchronous Audio-Video Visits for New Patient (98000 series)

Same categories and structure as Office or Other Outpatient codes 99202 99205 and 99211-99215.

Example (Taken from AMA) Description

99202 Office or other outpatient visit for the evaluation and management of a new patient, which requires a medically appropriate history and/or examination and straightforward medical decision making.

When using total time on the date of the encounter for code selection, 15 minutes must be met or exceeded.

#.98000 Synchronous audio-video visit for the evaluation and management of a new patient, which requires a medically appropriate history and/or examination and straightforward medical decision making.

When using total time on the date of the encounter for code selection, 15 minutes must be met or exceeded.

Synchronous Audio-Video Visits for New Patient (98000 series)

#● 98012

Synchronous audio-only visit for the evaluation and management of an established patient, which requires a medically appropriate history and/or examination, straightforward medical decision making, and more than 10 minutes of medical discussion.

When using total time on the date of the encounter for code selection, 10 minutes must be exceeded.

➔ *CPT Changes: An Insider's View 2025*

▶ (Do not report 98012 for home and outpatient INR monitoring when reporting 93792, 93793) ◀

▶ (Do not report 98012 when using 99374, 99375, 99377, 99378, 99379, 99380 for the same call[s]) ◀

▶ (Do not report 98012 during the same month with 99487, 99489) ◀

▶ (Do not report 98012 when performed during the service time of 99495, 99496) ◀

Synchronous Audio-Video Visits for Established Patients (98000 series)

CPT Codes	Description
98004	Synchronous audio-video visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination and straightforward medical decision making. When using total time on the date of the encounter for code selection, 10 minutes must be met or exceeded.
98005	Synchronous audio-video visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination and low medical decision making. When using total time on the date of the encounter for code selection, 20 minutes must be met or exceeded.
98006	Synchronous audio-video visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination and moderate medical decision making. When using total time on the date of the encounter for code selection, 30 minutes must be met or exceeded.
98007	Synchronous audio-video visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination and high medical decision making. When using total time on the date of the encounter for code selection, 40 minutes must be met or exceeded.

Synchronous Audio-Video Visits for Established Patients (98000 series)

CPT Codes	Description
98008	Synchronous audio-only visit for the evaluation and management of a new patient , which requires a medically appropriate history and/or examination, straightforward medical decision making, and more than 10 minutes of medical discussion . When using total time on the date of the encounter for code selection, 15 minutes must be met or exceeded.
98009	Synchronous audio-only visit for the evaluation and management of a new patient , which requires a medically appropriate history and/or examination, low medical decision making, and more than 10 minutes of medical discussion . When using total time on the date of the encounter for code selection, 30 minutes must be met or exceeded.
98010	Synchronous audio-only visit for the evaluation and management of a new patient , which requires a medically appropriate history and/or examination, moderate medical decision making, and more than 10 minutes of medical discussion . When using total time on the date of the encounter for code selection, 45 minutes must be met or exceeded.
98011	Synchronous audio-only visit for the evaluation and management of a new patient , which requires a medically appropriate history and/or examination, high medical decision making, and more than 10 minutes of medical discussion . When using total time on the date of the encounter for code selection, 60 minutes must be met or exceeded.

Synchronous Audio-Video Visits for Established Patients (98000 series)

CPT Codes	Description
98012	Synchronous audio-only visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination, straightforward medical decision making, and more than 10 minutes of medical discussion. When using total time on the date of the encounter for code selection, 10 minutes must be exceeded.
98013	Synchronous audio-only visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination, low medical decision making, and more than 10 minutes of medical discussion. When using total time on the date of the encounter for code selection, 20 minutes must be met or exceeded.
98014	Synchronous audio-only visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination, moderate medical decision making, and more than 10 minutes of medical discussion. When using total time on the date of the encounter for code selection, 30 minutes must be met or exceeded.
98015	Synchronous audio-only visit for the evaluation and management of an established patient , which requires a medically appropriate history and/or examination, high medical decision making, and more than 10 minutes of medical discussion. When using total time on the date of the encounter for code selection, 40 minutes must be met or exceeded.



Telehealth Consent is Mandatory

Telehealth Consent Forms

Telemedicine Verbal Consent and Financial Responsibility: Telemedicine enables healthcare providers at different locations to provide safe, effective, and convenient care through the use of technology. As with any health care service, there are risks associated with the use of telemedicine, including equipment failure, poor images, and information security issues. You have the right to withhold or withdraw your consent to use Telemedicine in the course of your care at any time. Patient/Patient's personal representative verbally consented to the use of telemedicine in their medical care.

Provider confirmed the following: Patient identifiers / Location of patient / Others present / Location of provider / Form of telemedicine (HIPAA-compliant platform or audio-only) with all risks to mode of healthcare delivery discussed.



Telehealth Consent is Mandatory

Telehealth Consent Forms

Financial Responsibility for Telemedicine: It has been explained, and you the patient/guarantor understand and have verbally accepted financial responsibility, for the Telemedicine Services provided. We may verify your insurance benefits and submit a claim to your health insurer for payment. Reimbursement for this service is dependent upon your carrier's terms and conditions. We make no representation nor assumption of liability that the services provided are an eligible reimbursable expense. Without waiving your financial responsibility, you agree to assign your right to medical benefits to the Health Care Professional who treated you. If your health insurer does not pay your claim, or only pays a portion of your claim, you agree to pay the remaining charges.



HIPAA – Compliant Platform is Mandatory

HIPAA-compliant platform is a must in 2025 and beyond.

HIPAA-compliant platforms include Doxy.me, Zoom for Healthcare, Skype for Business, Microsoft Teams, Google G-Suite Hangouts Meet.

High risk finding in 100% audits I have personally conducted.

Many waivers applied during the Covid-19 PHE
(that was 2020-2023!)

The Office of Civil Rights (OCR) allowed use of FaceTime and Skype, but advised against using public-facing application like Facebook Live and TikTok.

What's Wrong Here? - 99212

Type of telemedicine: audio, time spent in minutes: 5 min

Patient was identified using two of the following identifiers: name and DOB

Patient Location in California: home

Provider Location: office

Platform Used: Other



What's Wrong Here? - 99212

OPERATION: Robotic Assisted Cholecystectomy, Liver Cyst Fenestration, Repair Of Pre Existing Bile Leak, Vascularized Pedicle Flap, Intraoperative Ultrasound.

HPI: XXX is a 72 y.o. female with a long history of a symptomatic liver cyst and symptomatic cholelithiasis who underwent robotic assisted cholecystectomy, liver cyst fenestration and repair of pre-existing bile leak on 03/29/2024. In the operating room, was found to have a large liver cyst. On removing the cyst wall, was found to have pre-existing biliary communication with the segment 8 biliary branches that required suture ligation. A drain was placed and has since been removed as it was serous and no bile. She is overall recovering quite well from surgery and having no right upper quadrant pain or fevers or chills. She called in today wanting to discuss some change in bowel habits as well as large amount of flatulence.

REVIEW OF SYSTEMS: A fourteen point review of symptoms was performed. This was negative except for the pertinent positive and negatives which are noted in the HPI

PHYSICAL EXAM:

No vitals obtained as this was a brief phone call



What's Wrong Here? - 99212

Total time spent 5 min

Greater than 50% of the total time spent was face-to-face with the patient. The importance of treatment compliance and the goals of therapies were thoroughly reviewed. The risks, benefits, and alternatives for all treatment options were thoroughly discussed. In addition, instructions for managing therapy as well as instructions for follow-up, risk factor reduction, and patient and family education were explained in details. All patient questions were answered, both verbally, as well as with pertinent written patient information materials.

Medical decision making was high complexity. This patient's full medical records, diagnostic testing reports, past medical history, social history, family history, and review of systems, as well as overall clinical status, were reviewed and analyzed. Multiple treatment options have been considered, and the risk of treatment-related complications, morbidity, or mortality weighed carefully.



Telehealth Under Scrutiny



Telehealth Under TPE Scrutiny

CMS gave us breathing room for 2 years after 2021 E/M overhaul

TPE audits were active across several jurisdictions in 2023 for moderate and high-level office visits based on MDM or time – they also included services rendered via telehealth

TPE audits in 2025 include hospital/observation codes – they also include those services rendered via telehealth



Telehealth Under OIG Scrutiny

OIG Toolkit on Analyzing Telehealth Claims To Assess Program Integrity Risks

In the September 2022 data brief Medicare Telehealth Services During the First Year of the Pandemic: Program Integrity Risks, OEI-02-20-00720, OIG identified 1,714 providers whose billing for telehealth services during the first year of the pandemic poses a high risk to Medicare. As a follow-up to the data brief, OIG plans to release a toolkit that will provide information for our public and private sector partners about analyzing claims data for telehealth services. Gaining a better understanding of the program integrity risks associated with telehealth can help policymakers and stakeholders develop necessary safeguards and address individual cases of fraud, waste, and abuse.

What does the toolkit include?

The goal of this toolkit is to provide an approach to analyzing claims data for telehealth to identify areas in which additional safeguards may be necessary. The analysis can also help identify providers whose billing may pose a risk and warrant further scrutiny.

The toolkit includes detailed descriptions of seven data analysis measures that can be applied to the user's own data. Users can also modify the measures to meet their individual needs, such as identifying providers at varying levels of risk.



Program Integrity Risk

5 Fraud and Abuse Laws that could be triggered:

False Claims Act (FCA) - 31 U.S.C. §§ 3729–3733

Anti-Kickback Statute (AKS) - 42 U.S.C. § 1320a-7b(b)

Stark Law - 42 U.S.C. § 1395nn

Exclusion Statute - 42 U.S.C. § 1320a-7

Civil Monetary Penalties - 42 U.S.C. § 1320a-7a

Commercial payors also have watch dogs – the SIU



Other Payors



Other Payors – BCBS IL

Telemedicine and Telehealth/Virtual Health Care Services Policy

Policy Number: CPCP033

Version 1

Enterprise Clinical Payment and Coding Policy Committee Approval Date: February 20, 2025

Plan Effective Date: January 1, 2025



Other Payors – BCBS IL

Key billing requirements

Place of Service (POS) codes:

POS 10: Use for services provided in the patient's home.

POS 02: Use for services provided outside of the patient's home (e.g., a clinic).

Modifiers: Append the correct modifier to the CPT/HCPCS code.

Modifiers FQ, FR, Go, GT, 93, 95, and others: are used to identify the service type.

Modifier 95: is used for audio-visual telehealth, and modifier 93 is used for audio-only visits, with specific requirements depending on the payer.

Check the specific policy details to determine the correct modifier for your service.

Codes:

Use the new evaluation and management (E/M) telehealth codes, effective January 1, 2025.

Use the correct ICD-10 diagnosis codes and CPT procedure codes for your service.

If billing for the originating site facility fee, use HCPCS code Q3014 when applicable.

Other Payors – Cigna



Reimbursement Policy Commercial

Effective Date03/12/2020
Last Updated Date08/29/2025
Annual Review Date03/12/2026
Reimbursement Policy Number..... R31

Virtual Care



Other Payors - Cigna

General Requirements:

Telehealth services must be provided by a licensed healthcare professional in the United States.

Services must be performed using audio-video or audio-only technology.

Patients must be located in a place that is eligible for telehealth services under Medicare regulations.

Billing Codes:

Use the same Evaluation and Management (E&M) codes as for in-person visits (99202-99215).

Append modifier 95 for audio-video services.

Append modifier 93 for audio-only services.

Use Place of Service (POS) code 02 for telehealth services performed in a non-physician's office setting, such as a patient's home.

Use POS code 10 for telehealth services performed in a physician's office.



Other Payors - Cigna

Specific Considerations:

Mental Health Services: Telehealth mental health services are covered without geographic restrictions through September 30, 2025.

Chronic Disease Management: Telehealth services for chronic disease management, such as diabetes and heart failure, are covered.

Post-Acute Care: Telehealth services for post-acute care, such as physical therapy and occupational therapy, are covered.

Down-coding: Cigna may down-code telehealth services if they do not meet the criteria for the billed E&M code.



Final Thoughts



Final Thoughts

- **Policy Shift** - Huge shift back to telehealth BEFORE the pandemic for Medicare population
- **Government Shutdown Trickle Down Effect** - Practices are uncertain of next steps
- **States Fly Their Own Flags** – So much variance
- **Compliance** – What's That?
- **Due Diligence** – Continued responsibility
- **C-Suite** – Leadership decisions and strategic moves



Final Thoughts

- 100% failure rate in telehealth audits I am performing in 2025 for behavioral health, general, and specialty providers.
- Missed verbal consent for audio-only AND rationale why this is not audio-video.
- Missed HIPAA-compliant platform.
- Incorrect place of service codes for State and commercial payors.
- Missed telehealth modifiers for State and commercial payors.



Final Thoughts

- Outdated EMR templates cause unforced errors.
- Develop mandatory telehealth education and training in 2025.
- Create State payor telehealth matrices.
- Create commercial payor telehealth matrices.
- Are you ready to pivot for Medicare as of 11/5/2025?



Resources



Resources

- <https://www.reuters.com/world/us/us-government-shutdown-what-is-open-what-is-closed-2025-10-01/>
- <https://www.cms.gov/files/document/telehealth-faq-updated-10-15-2025.pdf>
- <https://www.ngsmedicare.com/web/ngs/news-article-details?selectedArticleId=14162185&redirecturl=%2Fweb%2Fngs%2Fnews%3Flob%3D96664%26state%3D97244%26region%3D93624&lob=96664&state=97244®ion=93624>
- <https://www.cms.gov/files/document/mln901705-telehealth-remote-patient-monitoring.pdf>
- <https://www.cms.gov/medicare/medicare-general-information/bni/downloads/abn-form-instructions.pdf>
- <https://codes.ohio.gov/ohio-administrative-code/rule-4725-25-01>
- <https://www.ilga.gov/Legislation/ILCS/Articles?ActID=3807&ChapterID=24>
- <https://www.cchpca.org/ohio/?category=medicaid-medicare&topic=overview>
- <https://www.cchpca.org/illinois/?category=medicaid-medicare&topic=overview>
- <https://www.aetna.com/individuals-families/health-insurance-through-work/telemedicine.html>
- <https://connect.bcbsil.com/my-coverage-explained/b/weblog/posts/care-online-virtual-visits>
- <https://www.uhc.com/member-resources/health-care-tools/telehealth-virtual-care>
- <https://www.cigna.com/individuals-families/member-guide/virtual-care-services>
- [CPT® Coding Manual 2025](#)
- <https://www.cms.gov/newsroom/fact-sheets/calendar-year-cy-2026-medicare-physician-fee-schedule-final-rule-cms-1832-f>
- <https://public-inspection.federalregister.gov/2025-19787.pdf>

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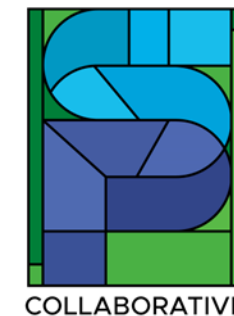
<https://www.linkedin.com/company/spcollaborative/>

Listen to Sonal on Paint The Medical Picture Podcast ::

<https://podcasts.apple.com/us/podcast/paint-the-medical-picture-podcast/id1530442177>

See where Sonal is Speaking next, services, and E-Tools ::

<https://spcollaborative.net/>



SP Collaborative, LLC Resources



Sonal Patel, BA, CPMA, CPC, ... • You

♦ CEO & Principal Strategist ♦ Healthcare Co...

3w • 🌐



#ThursdayThoughts 💬 💬

My thoughts this month center around what bravery means - even when facing difficulties.

👉 Show up for those that need you.

"When you are firmly rooted, you can fully stand up for others." — Rachel Wilkerson Miller

If you enjoyed this, share it 🔄 and follow 🙌 me, [Sonal Patel, BA, CPMA, CPC, CMC, ICDCM](#), for all my daily posts.

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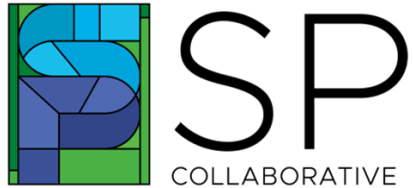
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Thank You



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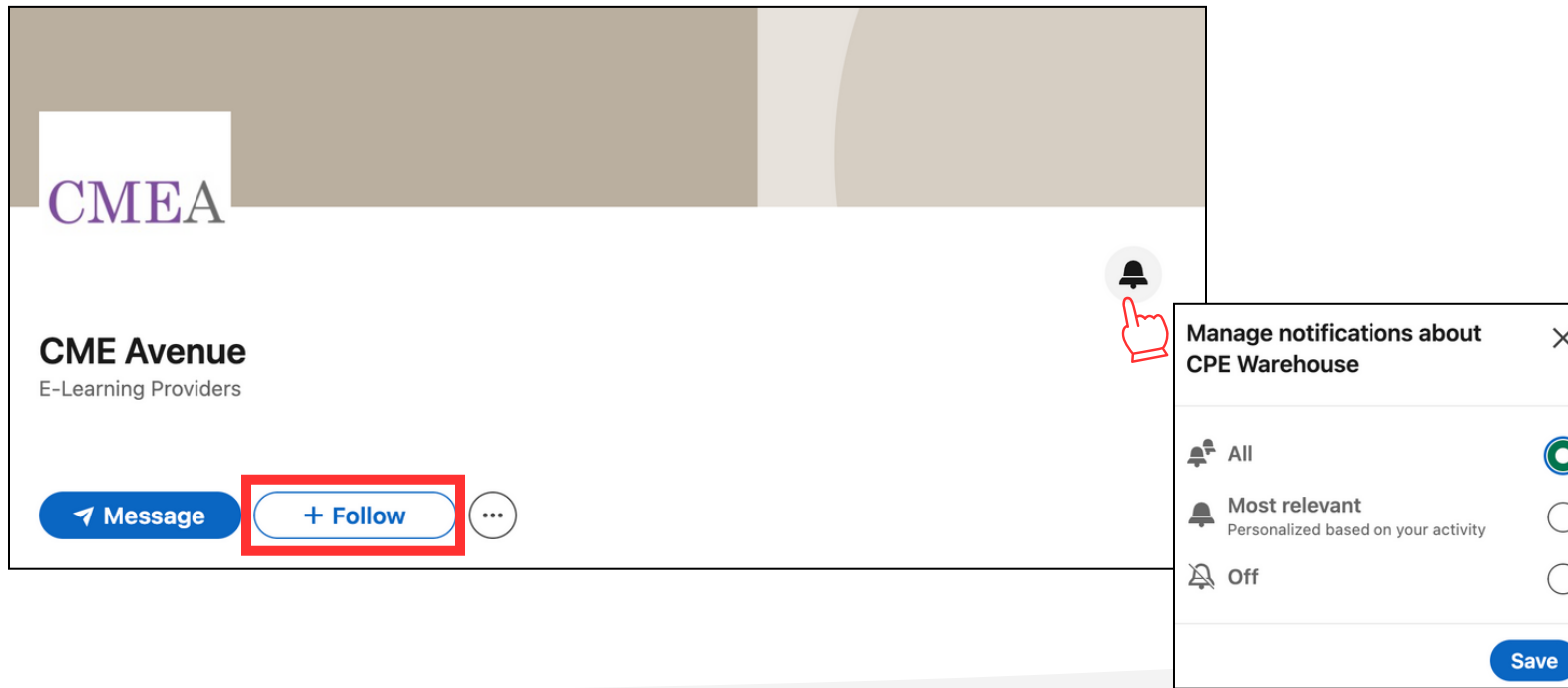
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